

VA EHR Operational Adoption Evidence Packet

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Blind-test question

Diagnose why Mann-Grandstaff clinical and administrative staff were not able to use the new electronic health record's core functions reliably after implementation.

Evidence boundary

This is an evidence-only reconstruction from dated records, attributed accounts, training artifacts, survey results, and operating data reproduced or described in VA OIG Report 20-01930-183. The OIG's findings, conclusions, causal labels, and recommendations are deliberately excluded.

The full OIG report is the held-out validation source. Do not consult it while diagnosing this packet.

Statements below identify whether they are a record, attributed account, or analyst limitation. Page references use the report's printed page numbers.

Artifact 1 - Deployment and operating outcome record

Record

- VA originally targeted March 28, 2020, for the first deployment at Mann-Grandstaff.
- On February 10, 2020, VA postponed the planned deployment when the system was described as 75-80 percent ready.
- VA resumed implementation activity in August 2020. During the intervening period, the program reported completing infrastructure-readiness work and the design, build, connectivity, and technical testing required for 73 interfaces.
- On September 30, 2020, the program executive told Congress that remaining key milestones would be completed before deployment.
- On October 24, 2020, clinical and administrative staff began using the system for live work.

Post-deployment observation

- Two to three months after deployment, an OIG survey asked staff about four core functions: finding relevant patient information, sharing patient information, navigating applications, and documenting patient care.
- Only 5 percent of applicable respondents answered affirmatively on all four functions.

Source: VA OIG 20-01930-183, pp. 5, 9, 11, and 13.

Artifact 2 - Formal responsibilities and training approval record

Record

- The VA Office of Electronic Health Record Modernization (OEHRM) was responsible for implementation of the new system, including training.
- Veterans Health Administration clinical and administrative personnel were the operational users expected to deliver care and perform daily work through the system.
- Cerner developed the training plans and materials. VA National Councils reviewed instructor-led content for alignment with validated workflow decisions.
- VA OEHRM then reviewed and approved the training materials and was responsible for checking alignment with training objectives, workflow, functionality, and audience.
- The training strategy made course completion and proficiency checks prerequisites for system access.

Source: VA OIG 20-01930-183, pp. 1, 5-6, and 10.

Artifact 3 - Operational-readiness activity record

December 2019 memorandum

- The Facility Director told VA OEHRM's Executive Director and Chief Medical Officer that the scheduled mock go-live would not provide a comprehensive real-world operational assessment.
- He requested at least three weeks of scenario-based assessment by facility staff to identify functional obstacles before deployment.

January 8-9, 2020 meeting and follow-up

- The Deputy Secretary convened VA, VHA, and VA OEHRM leaders.
- The Acting Under Secretary for Health wrote that the planned testing would not fully assess the system's capabilities, noted that some manual workarounds were more significant than anticipated, and endorsed a full operational assessment before deployment.
- He characterized the assessment as necessary for patient safety.

Scheduling outcome

- The operational assessment was placed on the schedule before facility staff had completed system training.
- The Facility Director decided it would not be useful to run the assessment with untrained staff.
- The requested assessment was not completed before the planned March 2020 deployment date.

Source: VA OIG 20-01930-183, pp. 37-38.

Artifact 4 - Super-user feedback and release-response record

February 3, 2020

- The Facility Director met with more than 30 super users and reported broadly negative feedback about the training.
- A clinical council co-chair separately wrote to senior VA OEHRM leaders that the training had left participants demoralized, distrustful, and less prepared to assist other users.

February 5, 2020

- VA OEHRM's Chief Medical Officer and the Facility Director met with 50 super users.
- The Chief Medical Officer also considered a post-training survey completed by 22 of the 81 super users who had finished training. Those respondents generally rated the training favorably.
- The 22 survey respondents represented 27 percent of the 81 trained super users.
- After considering survey results and observations from change-management personnel, the Chief Medical Officer and VA OEHRM Executive Director decided the training issues did not require delaying end-user training or deployment.

February 6-10, 2020

- The Facility Director told the Acting Under Secretary for Health that significant readiness concerns remained and recommended a delay of at least 90 days.
- Four days later, VA announced that deployment would be postponed.

Evidence limitation

- A contemporaneous performance assessment stated that training-attendance and completion data were delayed and could not be validated using VA systems, reducing confidence in the reporting available to leadership.

Source: VA OIG 20-01930-183, pp. 38-39.

Artifact 5 - Training design, workflow, and practice-environment record

Training plan

- The new system introduced more than 900 new workflows for facility users.
- The approved strategy required the 300-level instructor-led curriculum to incorporate workflow and process changes.
- Course sequences included software instruction, scenario-based activities, supplemental resources, and proficiency checks.

Attributed operational-user accounts

- Facility personnel described classroom instruction as concentrating on how to execute software actions without enough context for how the actions fit into daily clinical and administrative work.
- Personnel reported that practice-based questions were often deferred and not resolved before deployment.
- Some topics were moved to more than 200 job aids, tip sheets, and reference materials that were optional rather than required coursework.

Practice environment

- The training environment was finalized in May 2020, while production functionality continued to change afterward.
- Some production workflows and user roles were absent from the training environment.
- Access was restricted to scheduled classroom activities; general practice access was not widely available.

Role assignment

- Training assignments depended on mapping staff into more than 300 system roles.
- Facility personnel reported incorrect role assignments, multiple-role complications, staffing changes, limited instructions, and repeated scheduling corrections.

Source: VA OIG 20-01930-183, pp. 6, 10-20.

Artifact 6 - Post-deployment survey and operating-data record

Survey method

- The OIG invited 1,799 facility employees to participate two to three months after deployment.
- 809 employees responded, a 45 percent response rate.
- 722 respondents used the new system or required access to it for their work.

Core-function responses

- 62 percent disagreed or strongly disagreed that relevant patient information was readily available.
- 53 percent disagreed or strongly disagreed that they could share patient information with other clinicians without difficulty.
- 65 percent disagreed or strongly disagreed that they could navigate the system's applications without difficulty.
- 55 percent disagreed or strongly disagreed that they could document patient care without difficulty.
- Five percent answered affirmatively on all four functions.

Completed-appointment volumes

Compared with the same months in 2019, completed appointments were lower by:

- 30.7 percent in August 2020,
- 25.6 percent in September 2020,
- 42.1 percent in October 2020, and
- 41.5 percent in November 2020.

Limitations

- COVID-19 contributed to reduced appointment volume, so the operating data cannot attribute the entire decline to the system transition.
- The survey response rate was 45 percent and does not establish the experience of every employee.
- This packet does not establish whether software defects independently caused part of the operational failure.

Source: VA OIG 20-01930-183, pp. 9, 11, 13, and 40-41.

Source and construction note

Primary source archive:

U.S. Department of Veterans Affairs, Office of Inspector General. Training Deficiencies with VA's New Electronic Health Record System at the Mann-Grandstaff VA Medical Center in Spokane, Washington. Report 20-01930-183, July 8, 2021.

Official record: <https://www.oversight.gov/reports/training-deficiencies-vas-new-electronic-health-record-system-mann-g-randstaff-va-medical>

Official PDF: <https://www.oversight.gov/sites/default/files/documents/reports/2021-07/VAOIG-20-01930-183.pdf>

Construction rule: factual chronology, attributed accounts, quantitative results, documented responsibilities, and explicit evidence limitations were retained. OIG conclusions, recommendations, and language naming the root cause were removed. No fact from outside the cited report was added.